

Management of Thrombolytic-Ineligible Patients with Acute Ischemic Stroke or TIA

When a patient presents with acute stroke symptoms but is **not eligible for thrombolytic therapy** (e.g., outside treatment window, contraindications present, minor non-disabling symptoms, or TIA), management should shift toward **secondary prevention** and **supportive stabilization**.

1. Initial Evaluation

- **Confirm non-cardioembolic stroke or high-risk TIA** using clinical exam, non-contrast head CT, and vascular imaging.
- Assess NIHSS to guide severity and risk stratification.
- If symptoms resolve within minutes to hours, consider diagnosis of **TIA** (Transient Ischemic Attack).

2. Antiplatelet Therapy

For Minor Stroke (NIHSS ≤ 3) or High-Risk TIA (ABCD² ≥ 4):

Dual Antiplatelet Therapy (DAPT):

- **Aspirin 160–325 mg loading dose**, then **81 mg daily**
- **Clopidogrel 300–600 mg loading dose**, then **75 mg daily**
- Duration: **21 days**, then continue aspirin monotherapy

DAPT reduces early stroke recurrence risk after minor stroke or high-risk TIA, particularly within the first 7–14 days.

Notable Trials:

- **CHANCE Trial** (Wang et al., NEJM, 2013)

- **POINT Trial** (Johnston et al., NEJM, 2018)

After 21 days, clopidogrel should be stopped to reduce bleeding risk.

3. Blood Pressure Management

Permissive Hypertension:

- In the **acute phase**, BP can remain elevated to maintain cerebral perfusion.
- **Target:**
 - Avoid lowering BP unless **>220/120 mmHg**
 - If on antihypertensives at baseline, hold unless BP dangerously elevated

If BP must be reduced, lower by **no more than 15% in the first 24 hours**.

When to Treat BP in Stroke:

- Evidence of hypertensive emergency (e.g., aortic dissection, MI, pulmonary edema)
- Preparing for **thrombolysis** or **surgical intervention**

First-line agents (IV):

- **Labetalol, Nicardipine, Clevidipine**

4. Avoid the Following in Acute Phase:

- Full-dose anticoagulation (unless atrial fibrillation with clear indication)
- Statins until swallowing is assessed (can be resumed within 48–72 hours)
- Delayed workup (carotid imaging, echocardiogram, telemetry should begin ASAP)

5. Supportive Care and Monitoring

- Monitor neuro status, glucose (target 140–180 mg/dL), and oxygenation

- Swallow screen before oral intake
- Initiate DVT prophylaxis (SCDs first, then anticoagulation when safe)
- Encourage early rehab involvement

References

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